ONCHOFIN

TERBINAFINE TABLETS 125MG AND 250 MG

NAME OF DRUG PRODUCT:

TERBINAFINE TABLETS 125MG

TERBINAFINE TABLETS 250MG

STRENGTH: **125mg and 250mg.**

Compositions:

Terbinafine Tablets 125mg:

Each uncoated tablet contains:

Terbinafine Hydrochloride Ph. Eur.

Equivalent to Terbinafine 125mg.

Terbinafine Tablets 250mg:

Each uncoated tablet contains:

Terbinafine Hydrochloride Ph. Eur.

Equivalent to Terbinafine 250mg.

Description:

Terbinafine Tablets 125mg:

White to off white, round uncoated, biconvex beveled edged tablets having 'D' debossed on one side and '56' on the other side.

Terbinafine Tablets 250mg:

White to off white, round uncoated, biconvex beveled edged tablets with break line and 'D' debossed on one side and '74' on the other side.

Pharmacodynamics:

Terbinafine is an allylamine, which has a broad spectrum of antifungal activity. At low concentrations terbinafine is fungicidal against dermatophytes, moulds and certain dimorphic fungi. The activity versus yeasts is fungicidal or fungistatic depending on the species.

Terbinafine interferes specifically with fungal sterol biosynthesis at an early step. This leads to a deficiency in ergosterol and to an intracellular accumulation of squalene, resulting in fungal cell death. Terbinafine acts by inhibition of squalene epoxidase in the fungal cell membrane.

The enzyme squalene epoxidase is not linked to the cytochrome P450 system. Terbinafine does not influence the metabolism of hormones or other drugs.

Pharmacokinetics:

Following oral administration, terbinafine is well absorbed (>70%) and the bioavailability of terbinafine hydrochloride tablets as a result of first-pass metabolism is approximately 40%. Peak plasma concentrations of 1 mcg/mL appear within 2 hr. after a single 250 mg dose; the AUC (area under the curve) is approximately 4.56 mcg·h/mL. A single oral dose of 250mg terbinafine results in mean peak plasma concentrations of 0.97µg/ml within 2 hours after administration. The absorption half-life is 0.8 hours and the distribution half-life is 4.6 hours. Terbinafine binds strongly to plasma proteins. It rapidly diffuses through the dermis and concentrates in the lipophilic stratum corneum. In patients with renal impairment (creatinine clearance ≤ 50mL/min) or hepatic cirrhosis, the clearance of terbinafine is decreased by approximately 50% compared to normal volunteers. No effect of gender on the blood levels of terbinafine was detected. Terbinafine is also secreted in sebum, thus achieving high concentrations in hair follicles, hair and sebum rich skins. There is also evidence that terbinafine is distributed into the nail plate within the first few weeks of commencing therapy. Biotransformation results in metabolites with no antifungal activity, which is excreted predominantly in the urine. The elimination half-life is 17 hours. There is no evidence of accumulation. No age-dependent changes in pharmacokinetics have been observed but the elimination rate may be reduced in patients with renal or hepatic impairment, resulting in higher blood levels of terbinafine. The bioavailability of Terbinafine is unaffected by food. Terbinafine is distributed to the sebum and skin. A terminal half-life of 200-400 h may represent the slow elimination of terbinafine from tissues such as skin and adipose. Prior to excretion, terbinafine is extensively metabolized. No metabolites have been identified that have antifungal activity similar to terbinafine. Approximately 70%of the administered dose is eliminated in the urine.

Indications:

Onychomycosis (fungal infection of the nail) caused by dermatophyte fungi.

Tinea capitis

Fungal infections of the skin for the treatment of tinea corporis, tinea cruris, tinea pedis and yeast infections of the skin caused by the genus Candida (e.g. Candida albicans) where oral therapy is generally considered appropriate owing to the site, severity or extent of the infection.

Note: In contrast to Terbinafine topical, oral Terbinafine is not effective in pityriasis versicolor.

Recommended Dose:

The duration of treatment varies according to the indication and the severity of the infection.

Children:

No data are available in children under two years of age (usually <12 kg)

Children weighing < 20 kg 62.5 mg (half a 125 mg tablet) once daily

Children weighing 20 to 40 kg 125 mg (one 125 mg tablet) once daily

Children weighing > 40 kg 250 mg (two 125 mg tablets) once daily

Adults

250 mg once daily

Skin infections

Recommended duration of treatment

Tinea pedis (interdigital, plantar/moccasin type): 2 to 6 weeks

Tinea corporis, cruris: 2 to 4 weeks

Cutaneous candidiasis: 2 to 4 weeks

Complete resolution of the signs and symptoms of infection may not occur until several weeks after mycological cure.

Hair and scalp infections

Recommended duration of treatment:

Tinea capitis: 4 weeks

Tinea capitis occurs primarily in children

Onychomycosis

For most patients the duration of successful treatment is 6-12 weeks

Fingernail onychomycosis

Six weeks of therapy is sufficient for fingernail infections in most cases.

Toenail onychomycosis

Twelve weeks of therapy is sufficient for toenail infections in most cases. Some patients with poor nail outgrowth may require longer treatments. The optimal clinical effect is seen some months after mycological cure and cessation of treatment. This is related to the period required for outgrowth of healthy nail.

Use of Onchofin in the elderly

There is no evidence to suggest that elderly patients require different dosages or experience different side effects than younger patients. When prescribing tablets for patients in this age group, the possibility of pre-existing impairment of liver of kidney function should be considered

Use of Onchofin in the children

In children above 2 years of age, oral Onchofin has been found to be well tolerated.

Mode of administration:

Oral route of administration.

Contraindication

Terbinafiine hydrochloride tablets are contraindicated in patients with hypersensitivity to Terbinafine hydrochloride or any other ingredients of the formulation.

Warning and precautions

Onchofin is not recommended for patients with chronic or active liver disease. Before prescribing Onchofin tablets, pre-existing liver disease should be assessed.

Hepatotoxicity may occur in patients with and without pre-existing liver disease. Patients prescribed Onchofin tablets should be warned to report immediately any symptoms of unexplained persistent nausea, anorexia, fatigue, vomiting, right upper abdominal pain, or jaundice, dark urine or pale stools. Patients with these symptoms should discontinue taking oral terbinafine and patients's liver functions should be immediately evaluated.

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A/s : 125 x 260 mm ■ Black

AUROBINDO Packaging Development	Product Name: Onchofin	Component: Leaflet	Item Code: P1518886	Date & Time: 05.07.2018 & 9:40 am
	Customer / Country: Malaysia	Version No.: 00	Reason Of Issue: Revision	Reviewed / Approved by
Team Leader: Lova Raju	Dimensions: 125 x 260 mm	No. of Colours : 01		
Initiator: Anuradha				
Artist: Vision Graphic Designers	Pharmacode: 18886	18886		
Additional Information : Supersede Item Code: P1514346				Sign / Date

Patients with impaired renal function (creatinine clearance less than 50 ml/min or serum creatinine of more than 300 micro mol/l) should receive half the normal dose.

In vitro and in vivo studies have shown that terbinafine inhibits the CYP2D6 metabolism. Therefore, patients receiving concomitantly treatment with drugs predominantly metabolized by this enzyme, e.g. certain members of the following drug classes, tricyclic antidepressants (TCSs), β-blockers, selective serotonine reuptake (SSRIs), antiarrhythmics class 1C and monoamine oxidase inhibitors (MAO-Is) Type B, should be followed up, if the co-administered drug has a narrow therapeutic window.

Interactions with other medicaments:

Effect of other medicinal products on terbinafine

The plasma clearance of terbinafine may be accelerated by drugs, which induce metabolism and may be inhibited by drugs, which inhibit cytochrome P450, where co-administration of such agents is necessary, the dosage of Onchofin may need to be adjusted accordingly.

The following medicinal products may increase the effect or plasma concentration of terbinafine

Cimetidine decreased the clearance of terbinfaine by 33%

The following medicinal products may decrease the effect or plasma concentration of terbinafine

Rifampicin increased the clearance of terbinafine by 100%.

Effect of terbinfaine on other medicinal products

According to the results from studies undertaken in vitro and in healthy volunteers, terbinafine shows negligible potential for inhibiting or enhancing the clearance of most drugs that are metabolized via the cytochrome P450 system (e.g. terfenadine, triazolam, tolbutamide or oral contraceptives) with exception of those metabolished through CYP2D6 .

Terbinafine does not interfere with the clearance of antipyrine or digoxin.

Some cases of menstrual irregularities have been reported in patients taking Onchofin concomitantly with oral contrac-eptives, although the incidence of these disorders remains within the background of patients taking oral contraceptive alone.

Terbinafine may increase the effect or plasma concentration of the following medicinal products

Caffeine

Terbinafine decreased the clearance of caffeine administered intravenously by 19%.

Compounds predominately metabolished by CYP2D6

In vitro and in vivo studies have shown that terbinafine inhibits the CYP2D6-mediated metabolism. This finding may be of clinical relevance for compounds predominantly metabolished by this enzyme, e.g. certain members of the following drug classes, tricyclic antidepressants (TCAs), β-blockers, selective serotonine reuptake (SSRIs), antiar-rhythmics class 1C and monoamine oxidase inhibitors (MAO-Is) Type B, and if they also have a narrow therapeutic window.

Terbinafine decreased the clearance of desipramine by 82%.

Terbinafine may decrease the effect or plasma concentration of the following medicinal products

Terbinafine increased the clearance of ciclosporin by 15%.

Pregnancy and lactation

Pregnancy

Foetal toxicity and fertility studies in animals suggest no adverse effects. Since clinical experience in pregnant women is very limited, Onchofin should not be used during pregnancy unless the potential benefits outweigh any potential risks.

Lactation

Terbinafine is excreted in breast milk, mothers receiving oral treatment with Onchofin should therefore not breast-feed.

Side effects

In general Onchofin tablets are well tolerated. Side effects are usually mild to moderate and transient.

Adverse reactions (Table 1) are ranked under heading of frequency, the most frequent first, using the following convention: very common (≥ 1/10); common (≥ 1/100, < 1/10); uncommon (≥ 1/1,000, <1/100); rare (≥1/10,000, < 1/1,000); very rare (< 1/10,000), including isolated reports.

Table 1

Blood and lymphatic system disorders
Very rare: Neutropenia, agranulocytosis, throm ocytopenia.
Immune system disorders
Very rare: Anaphylactoid reactions (including angioedema), cutaneous and systemic lupus erythem-atosus.
Nervous system disorders
Common: Headache
Uncommon: Taste disturbances, including taste loss, which usually recover within several weeks after discontinuation of the drug isolated cases of prolonged taste disturbances, have been reported. A decrease of food intake leading to significant weight loss we observed in very few severe cases.
Hepatobiliary disorders
Rare: Hepatobiliary dysfunction (primarily cholestatic in nature), including very rare cases of serious liver failure (some with a fatal outcome, or requiring liver tansplant). In the majority of liver failure cases the patients had serious underlying systemic conditions and a causal association with the intake of Onchofin was uncertain.
Gastrointestinal disorders
Very common: Gastrointestinal symptoms (feeling of fullness, loss of appetite, dyspepsia, nausea, mild abdominal pain, diarrhoea)
Skin and subcutaneous tissue disorders
Very common: Non-serious forms of skin reactions (rash, urticaria)
Very rare: Serious skin reactions (e.g. Stevens-Johnson syndrome, toxic epidermal necrolysis, and acute generalized exanthematous pustulosis)*. Psoriasiform eruptions or exacerbation of psoriasis. Hair loss, although a causal relationship has not been established.
Musculoskeletal and connective tissue disorders
Very common: Musculoskeletal reactions (arthralgia, myalgia)
General disorders
Very rare: Fatigue

* If progressive skin rash occurs, Onchofin treatment should be discontinued.

Symptoms and Treatment of over dose

A few cases of overdosage (up to 5 g) have been reported, giving rise to headache, nausea, epigastric pain and dizziness.

The recommended treatment of overdosage consists of eliminating the drug, primarily by the administration of activated charcoal, and giving symptomatic supportive therapy, if needed.

Storage condition:

Store below 30º C. Protect from light

Shelf life:

24 months

Presentation:

125mg and 250mg: 2 blisters of 7 tablets each are packed in a printed carton along with a package insert.

Manufactured in India by:

AUROBINDO

Aurobindo Pharma Limited, Unit - III,
Sy. No. 313 & 314,
Bachupally, Bachupally Mandal,
Medchal-Malkajgiri District,
Telangana State, India.

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